# ENT Examination — OSCE Checklist

Deakin MD2 · BB4 · Time: 8 min (+2 min reading)

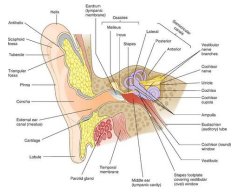
*Tick each step as you perform or verbalise it.* **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Ears (inspect → palpate → otoscopy → hearing) → Nose (inspect → airflow → rhinoscopy → sinuses) → Throat (mouth → tonsils → neck)**. *Deakin quick reference order. Examine the* **good ear first**.

## 1. Introduction

| | |
|---|---|
| ☐ | Wash hands; introduce self; confirm patient ID |
| ☐ | Explain the exam and gain consent; ask about pain |
| ☐ | Ask about ear, nose and throat symptoms; position seated, at your eye level |
| ☐ | Equipment: otoscope with range of speculae, 512 Hz tuning fork, pen torch, tongue depressor, nasal speculum (or otoscope with wide speculum), gloves |

## 2. Ears

| | Step | What to do / say | Abnormal |
|---|---|---|---|
| **Ear — inspection (external ear)** | | | |
| ☐ | Pinna | Size, shape, deformity; trauma (haematoma) | Cauliflower ear |
| ☐ | Pre-auricular region | Cartilage tags, sinuses or pits | Syndromes with hearing loss |
| ☐ | Skin | Suspicious lesions (SCC, BCC), skin conditions of pinna and canal | Neoplasia, eczema |
| ☐ | Scars | Pre- or post-auricular | Previous surgery |
| ☐ | Hearing aids | Note and remove before otoscopy | |
| ☐ | Discharge | Mucopurulent, serous, blood-stained | Otitis externa / media, perforation |



Anatomy of the ear




Otitis externa




Squamous cell carcinoma of the pinna



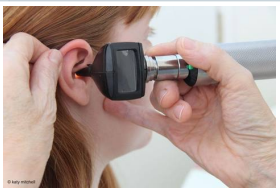

Preauricular sinus

| | | | |
|---|---|---|---|
| **Ear — palpation** | | | |
| ☐ | Tragus and pinna | Gently move — tenderness? | Tender = otitis externa |
| ☐ | Mastoid | Palpate behind the ear | Tender = mastoiditis (with otitis media) |

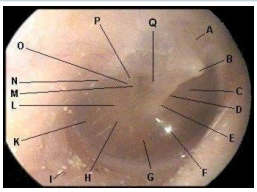



Acute mastoiditis

| | | | |
|---|---|---|---|
| **Ear — otoscopy** | | | |
| ☐ | Speculum | Largest that fits comfortably | |
| ☐ | Grip | Hold otoscope like a pen; hand braced against patient's head | Prevents injury if head moves |
| ☐ | Straighten canal | Adult: pull pinna up and back; child: pull back only | |
| ☐ | Canal | Inflamed? Wax? Discharge? Foreign body? | Otitis externa |
| ☐ | Tympanic membrane | Colour, light reflex, short process and handle of malleus, umbo, pars flaccida; move to see whole TM | Bulging, retraction, perforation, grommet |



Holding the otoscope — hand braced on head




Right tympanic membrane — landmarks



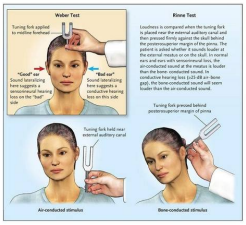
Grommet (ventilation tube)



Common ear drum findings

| | | | |
|---|---|---|---|
| **Ear — hearing** | | | |
| ☐ | Whisper test | Mask other ear (rub tragus / fingers); whisper numbers; increase volume if needed; each side | Formal audiology if abnormal |
| ☐ | Weber (512 Hz) | Strike fork; base on vertex or midline forehead: "Louder on one side or the middle?" | Lateralises (see table) |

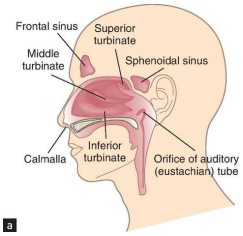
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| ☐ | Rinne (512 Hz) | Base on mastoid until no longer heard, then prongs beside the ear: "Can you hear it now?" | Positive (AC > BC) = normal |



Weber and Rinne tests

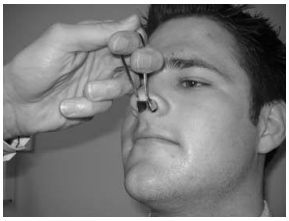
## 3. Nose and sinuses

| | Step | What to do / say | Abnormal |
|---|---|---|---|
| **Nose — external inspection** | | | |
| ☐ | Front and side | Swelling, deformity, scars or creases, redness / skin disease | Trauma, rhinophyma, BCC |
| ☐ | From above | Patient tilts head back; look down the bridge | Deviated nose |
| ☐ | Discharge | Rhinorrhoea, crusting; unilateral? | Unilateral offensive in a child = foreign body |
| ☐ | Nasal bones | Gently palpate bridge | Tenderness, crepitus, step = fracture |




Nasal and sinus anatomy

| | | | |
|---|---|---|---|
| **Nose — airflow** | | | |
| ☐ | Misting test | Cold metal tongue depressor under nose as patient breathes out | Condensation under both nostrils = airflow |
| ☐ | Sniff test | Occlude one nostril, ask patient to sniff; repeat | Unilateral obstruction |
| **Nose — anterior rhinoscopy (torch ± speculum)** | | | |
| ☐ | Without speculum | Tip nose up with thumb; look into the anterior nares with torch | |
| ☐ | With speculum / otoscope | Gently open; look straight back, not up | |
| ☐ | Septum | Straight or deviated? Bleeding points, clots, crusting, perforation; septal haematoma | Little's area = common epistaxis site †; haematoma = urgent drainage |
| ☐ | Turbinates | Inflammation, swelling, colour | Rhinitis (pale, boggy = allergic †) |
| ☐ | Polyps* | Grey, glistening, insensate † | Chronic rhinosinusitis |



Nasal speculum examination

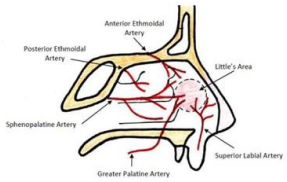



Figure 1. Blood supply to the nasal septum

Blood supply of the septum — Little's area

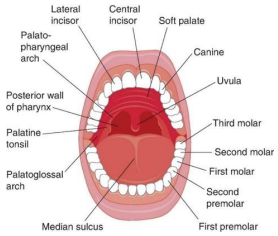


Nasal polyp

| | | | |
|---|---|---|---|
| **Sinuses** | | | |
| ☐ | Palpate | Frontal (above brow) and maxillary (cheek) sinuses | Tenderness = sinusitis |
| ☐ | Transilluminate* | Pen torch in a dark room (demonstrated in session) | Reduced = fluid / mass |
| ☐ | Note | Ethmoid and sphenoid sinuses cannot be examined physically | |

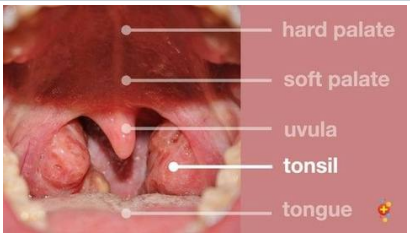
## 4. Throat and neck

| | Step | What to do / say | Abnormal |
|---|---|---|---|
| **Mouth — oral cavity (bright pen torch, mouth wide open)** | | | |
| ☐ | Lips and mucosa | Ulcers, lesions, colour | Aphthous ulcers, candida, malignancy † |
| ☐ | Dentition | Comment on teeth and gums | Dental caries, abscess |
| ☐ | Tongue | Appearance; ask to raise tongue to check underneath if indicated | Lesions, floor-of-mouth swelling |



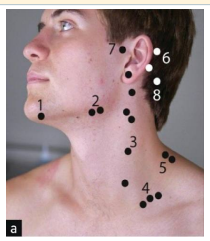

Anatomy of the mouth

| Throat — oropharynx | | | |
|---|---|---|---|
| ☐ | "Say ahh" | Mouth wide, tongue protruded | Palate rises symmetrically; uvula central † |
| ☐ | Tongue depressor | If tonsils not seen: press middle third of tongue, not the back † | |
| ☐ | Tonsils | Size, symmetry, inflammation, exudate | Tonsillitis; asymmetry = quinsy or malignancy † |
| ☐ | Pharynx | Swelling, inflammation, exudate, post-nasal drip | |




Palate, uvula and tonsils

| Neck (if clinically indicated) | | | |
|---|---|---|---|
| ☐ | Lymph nodes | From behind: submental, submandibular, tonsillar, cervical chain, supraclavicular, pre/post-auricular, occipital | Infection, malignancy |
| ☐ | Parotids* | Palpate in front of ear and over angle of jaw | Parotitis, tumour |




Cervical lymph node groups

## 5. Closing and presenting

| | |
|---|---|
| ☐ | Thank patient; clean otoscope speculum; dispose of tongue depressor; wash hands |
| ☐ | Present: "Pinnae normal and non-tender; canals clear; both TMs intact and pearly grey; whisper test normal, Weber central, Rinne positive bilaterally. No nasal deformity, airways patent, septum midline, no sinus tenderness. Good dentition; tonsils symmetrical without exudate; uvula central; no cervical lymphadenopathy." |
| ☐ | Completion: "I would arrange formal audiometry and consider nasendoscopy or a throat swab if indicated." |

## 6. Quick reference

| Weber / Rinne (Deakin table) | Weber central | Weber lateralises LEFT | Weber lateralises RIGHT |
|---|---|---|---|
| Rinne AC > BC both ears | Normal | Sensorineural loss RIGHT | Sensorineural loss LEFT |
| Rinne BC > AC left | — | Conductive loss LEFT | Combined loss |
| Rinne BC > AC right | — | Combined loss | Conductive loss RIGHT |

| TM appearance † | Think of |
|---|---|
| Pearly grey, light reflex | Normal |
| Red, bulging, lost landmarks | Acute otitis media |
| Dull, retracted ± fluid level / bubbles | Otitis media with effusion |
| Hole in TM | Perforation |
| Small tube in TM | Grommet (not a foreign body) |
| Swollen canal, debris, tender tragus | Otitis externa |

- **Conductive** loss: Weber goes TO the bad ear, Rinne negative (BC > AC).
- **Sensorineural** loss: Weber goes AWAY from the bad ear, Rinne positive (both reduced).
- **False-negative Rinne**: a dead ear hears BC louder via the other cochlea — Weber clarifies.
- **Quinsy †**: unilateral tonsil swelling, uvula deviated away, trismus, "hot potato" voice — urgent ENT.
- **Airway red flags †**: stridor, drooling, can't swallow saliva — don't use a tongue depressor; call for help.